

CC-002: Tom Bradley — Interview Transcript

Interview Transcript

PROJECT INFORMATION

Project	Home-Care-AI	Stage	Discovery → Stage 1 (Customer Research)
Artifact	2 (Synthetic Interview Data)	Participant ID	CC-002
Persona	Care Coordinator	Next Step	Feed into `/summarize-interview` with other CC interviews

SESSION DETAILS

Date	2026-04-01	Duration	36 min
Interviewer	PM Lead	Setting	Face-to-face
Location	HomeFirst Care office (small, one-room)		

PART A — Narrative Transcript

Opening & Warm-Up

Q: Tell me about your role --- what does a typical week look like?

I'm the only coordinator at HomeFirst. Twenty-five clients, six field staff. I do intake, scheduling, family communication, rostering, quality assurance, and — when nobody else is available — I fill in as a care worker myself. Last month I did that three times. My job title says coordinator but my actual role is 'person who does whatever needs doing.'

Block 1 --- Staffing Disruptions

Q: Walk me through the last time you lost a client.

Three months ago. The Hendersons. Mrs. Henderson is eighty-one, moderate dementia. Her regular carer went on leave, the backup got sick, and I had to send three different people in two weeks. Mrs. Henderson didn't recognise any of them. She was distressed. She kept asking where Carol was. Her daughter called me and said, 'My mother doesn't know who's walking through her door anymore.' They moved her to another agency the following week.

[Probe] Could you have prevented it?

If I'd had a system that automatically found the best available replacement — someone Mrs. Henderson had met before, or at least someone with dementia experience who'd been briefed on her specific needs — maybe. But I'm in Google Sheets. When someone calls sick, I scroll the sheet, mentally filter for who's qualified, who's available, who lives nearby, and start texting. Sometimes I find someone in ten minutes. Sometimes it takes an hour. And about twenty percent of the time, I can't find anyone and I have to cancel entirely.

Block 3: Communication

Q: When a visit gets cancelled, how do families find out?

This is the part I'm ashamed of. Sometimes they find out because nobody shows up. A family called me at nine AM asking why nobody came for their father's eight AM visit. I didn't even know it was cancelled. The carer had texted me at six AM but I was dealing with another fire. Nobody told the family. Their father had

been sitting in his chair, dressed and ready, waiting. That image stays with me.

[Emotional note] Tom's voice dropped. This clearly haunts him.

Block 5: Willingness to Prioritize

Q: If a system could handle vacant visit automation, what would change?

Everything. I spend thirty to sixty minutes per absence on phone calls, texts, and schedule juggling. If the system detected the absence, found replacements ranked by match quality, sent me one approval click, and then notified the carer, the client, and the family — I'd have my mornings back. I'd probably keep clients we're currently losing. And nobody's father would sit in a chair waiting for someone who isn't coming.

Q: Are there care plans in your system that are overdue for review?

Almost certainly. The quality framework says every eight weeks. I don't have a system that flags when they're due. I have a calendar reminder that I snooze more often than I act on. I can feel things slipping through but I can't see them. It's a vague anxiety with no specifics.

PART B — Filled Note Template

Participant ID	CC-002
Persona	CC
Date	2026-04-01
Duration (min)	36
Interviewer(s)	PM Lead
KEY JOBS	Schedule 25 clients across 6 staff. Handle absences (sole coordinator). Maintain client relationships and continuity. Quality assurance. Fill in as care worker when needed.
CURRENT SOLUTION	Google Sheets for scheduling. WhatsApp for staff comms. Calendar reminders (frequently snoozed). Paper forms for some clients. Himself as backup care worker. "A family called at 9 AM asking why nobody came. The carer texted me at 6 AM but I was dealing with another fire. Nobody told the family." "I know there are care plans overdue for review. I can feel it. But I don't have a list, I don't have a flag, just vague anxiety."
WILLINGNESS	30-60 min per absence. 20% of vacant visits end in cancellation (lost revenue + client risk). Lost Henderson family = lost revenue + reputation damage. Agency would immediately invest.
EMOTIONAL INTENSITY	Voice dropped describing father waiting in chair. Shame about uninformed cancellations. 'Vague anxiety' about compliance gaps. Self-deprecating about wearing too many hats.
ESCALATION BEHAVIOR	Tom is a one-person operation. No escalation chain — he IS the chain. When overwhelmed, visits get cancelled without notification. Quality documentation slips first when fires are burning.
SURPRISE FINDING	20% cancellation rate for vacant visits — quantifiable revenue and safety loss. Also: care plan review overdue problem is universal but invisible (no flags, no lists, just 'vague anxiety').
TOOL MENTIONS	Google Sheets. WhatsApp. Calendar reminders (snoozed). Paper forms.
REFERRAL	Angela Morrison (larger agency, same problems at bigger scale).

FOLLOW-UP ACTIONS

[] Quantify vacancy-to-cancellation rate across agencies [] Overdue care plan flagging as compliance feature [] Client loss attribution to continuity failures